United Healthcare

UnitedHealthcare Commercial

Medical Policy Update Bulletin: June 2025

In This Issue

Medical Policy Updates — Page

Updated

- Breast Reduction Surgery – Effective Jun. 1, 2025 3
- Category III Codes – Effective Jul. 1, 2025 3
- Home Health, Skilled, and Custodial Care Services (for Commercial Only) – Effective Jul. 1, 2025 3
- Manipulative Therapy – Effective Jun. 1, 2025 3
- Ocular Photoscreening – Effective Jun. 1, 2025 3
- Surgery of the Knee – Effective Jun. 1, 2025 3

Revised

- Chelation Therapy – Effective Jul. 1, 2025 4
- Epidural Steroid Injections for Spinal Pain – Effective Jul. 1, 2025 4
- Gastrointestinal Motility Disorders, Diagnosis and Treatment – Effective Aug. 1, 2025 5
- Gynecomastia Surgery – Effective Jul. 1, 2025 7
- Hyperbaric Oxygen Therapy and Topical Oxygen Therapy – Effective Jul. 1, 2025 8

Retired

- Lithotripsy for Salivary Stones – Effective Jun. 1, 2025 9
- Unicondylar Spacer Devices for Treatment of Pain or Disability – Effective Jun. 1, 2025 9

Medical Benefit Drug Policy Updates

Revised

- Amondys 45® (Casimersen) – Effective Jul. 1, 2025 10
- Clotting Factors, Coagulant Blood Products, & Other Hemostatics – Effective Jul. 1, 2025 11
- Entyvio® (Vedolizumab) – Effective Jul. 1, 2025 15
- Exondys 51® (Eteplirsen) – Effective Jul. 1, 2025 17
- Korsuva® (Difelikefalin) – Effective Jul. 1, 2025 19
- Maximum Dosage and Frequency – Effective Jul. 1, 2025 20
- Oncology Medication Clinical Coverage – Effective Jul. 1, 2025 23

In This Issue

- Oxlumo® (Lumasiran) and Rivfloza® (Nedosiran) – Effective Jul. 1, 2025 25
- Spevigo® (Spesolimab-Sbzo) – Effective Jul. 1, 2025 29
- Spevigo (Spesolimab-Sbzo) – IV and Subcutaneous Formulations (for UHCWest Only) – Effective Jul. 1, 2025 33
- Testosterone Replacement or Supplementation Therapy – Effective Jul. 1, 2025 37
- Ustekinumab – Effective Jul. 1, 2025 44
- Viltepso® (Viltolarsen) – Effective Jul. 1, 2025 51
- Vyondys 53® (Golodirsen) – Effective Jul. 1, 2025 53
- White Blood Cell Colony Stimulating Factors – Effective Jul. 1, 2025 55

Retired

- Zulresso® (Brexanolone) – Effective Jun. 1, 2025 57

Updated		
Policy Title	**Effective Date**	**Summary of Changes**
Breast Reduction Surgery	Jun. 1, 2025	**Related Policies *and* Applicable Codes** • Removed reference link to the Medical Policy titled *Panniculectomy and Body Contouring Procedures* **Benefit Considerations** • Added language to indicate specific exclusions for breast reduction surgery would not apply to the treatment of gender dysphoria where plans have a gender dysphoria treatment benefit **Supporting Information** • Updated *References* section to reflect the most current information
Category III Codes	Jul. 1, 2025	**Applicable Codes** • Added CPT codes 0623T, 0624T, 0625T, 0626T, 0948T*, 0949T*, 0950T*, 0956T*, 0957T*, 0958T*, 0959T*, 0960T*, 0962T*, 0963T*, 0967T*, 0968T*, 0969T*, 0970T*, 0971T*, 0972T*, 0973T*, 0974T*, 0975T*, 0976T*, 0977T*, 0978T*, 0979T*, 0980T*, 0981T*, 0982T*, and 0983T* *(*quarterly edit)*
Home Health, Skilled, and Custodial Care Services (for Commercial Only)	Jul. 1, 2025	**Related Policies** • Added reference link to the Medical Policy titled *Chelation Therapy* **Definitions** • Updated definition of “Respite Care” **Applicable Codes** • Removed CPT code 99512 **Benefit Considerations** • Removed language indicating a written treatment plan must be submitted with the request for specific services and supplies; periodic review of the written treatment plan may be required for continued skilled care needs and progress toward goals
Manipulative Therapy	Jun. 1, 2025	**Definitions** • Added definition of “Upledger Technique” **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information
Ocular Photoscreening	Jun. 1, 2025	**Benefit Considerations** • Added language to clarify most UnitedHealthcare Commercial and Individual Exchange plans cover instrument-based screenings (CPT codes 99174 and 99177) *as a preventive care services benefit* in certain circumstances **Supporting Information** • Updated *Clinical Evidence* section to reflect the most current information
Surgery of the Knee	Jun. 1, 2025	**Definitions** • Updated definition of “Reduced Allograft Discs [e.g., Cartiform (Arthrex), ProChondrix CR (Allosource)]” **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Chelation Therapy	Jul. 1, 2025	**Title Change** • Previously titled *Chelation Therapy for Non-Overload Conditions* **Coverage Rationale** • Replaced language indicating "chelation for heavy metal toxicity and overload conditions (e.g., iron, copper, lead, aluminum) is proven and medically necessary *and not addressed in this policy*" with "chelation for heavy metal toxicity and overload conditions (e.g., iron, copper, lead, aluminum) is proven and medically necessary" **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information	**Chelation for heavy metal toxicity and overload conditions (e.g., iron, copper, lead, aluminum) is proven and medically necessary.** **The following are unproven and not medically necessary due to insufficient evidence of efficacy:** • Chelation therapy for treating any chronic, progressive diseases associated with non-overload conditions • Chelation therapy for treating mercury "toxicity" from dental amalgam fillings

Epidural Steroid Injections for Spinal Pain	Jul. 1, 2025	**Coverage Rationale**
		• Revised coverage criteria for Epidural Steroid Injections (ESI); replaced criterion requiring "there is evidence of structural and/or functional nerve root involvement by imaging or *electromyography (EMG)*" with "there is evidence of structural and/or functional nerve root involvement by imaging or *electrodiagnostic studies*"
		Supporting Information
		• Updated *Clinical Evidence* and *References* sections to reflect the most current information
			Epidural Steroid Injections (ESI) are proven and medically necessary when all of the following criteria are met:
			• The injection is intended for the management of Radicular Pain as evidenced by history and physical exam; and
			• The Radicular Pain is unresponsive to the following conservative treatment for ≥ 4 weeks:
			  ○ Pharmacotherapy such as NSAIDS or acetaminophen; or
			  ○ Activity modification (including but not limited to heavy lifting, bending, spinal torsion activities); or
			  ○ PT or home exercise; and
			• There is evidence of structural and/or functional nerve root involvement by imaging or electrodiagnostic studies; and
			• The injection is performed under fluoroscopic or CT guidance
			
			Conditions that would contraindicate ESIs include but are not limited to:
			• Spinal neoplasm

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**

Epidural Steroid Injections for Spinal Pain
(continued)
	Jul. 1, 2025		• Rapidly progressing neurological deficit
			• Epidural abscess
			
			The following are unproven and not medically necessary due to insufficient evidence of efficacy:
			• The use of ultrasound guidance for ESIs
			• ESI for all other indications of the spine not included above
			
			Epidural Steroid Injection Limitations:
			• A maximum of four (4) ESI sessions (per region, regardless of level, location, or side) per year
			  ○ A session is defined as one date of service in which ESI injection(s) are performed
			  ○ A region is defined by either the region of the cervical, thoracic, or lumbosacral
			  ○ A year is defined as the 12-month period starting from the date of service of the first approved injection
			• Subsequent ESIs may be provided only if:
			  ○ Radicular Pain has returned and/or deterioration in function has occurred; and
			  ○ One of the following:
			  ▪ The previous injection resulted in ≤ 50% pain relief or functional improvement for less than three months as measured by validated measurement tools and there has been a reassessment of the individual and the injection site and technique; or
			  ▪ The previous injection resulted in ≥ 50% pain relief or functional improvement for three or more months as measured by validated measurement tools

Gastrointestinal Motility Disorders, Diagnosis and Treatment	Aug. 1, 2025	**Coverage Rationale** • Added language to indicate Esophageal Mucosal Integrity Testing by electrical impedance (e.g., MiVu™ Mucosal Integrity Testing System) for the diagnosis of gastroesophageal reflux disease (GERD), eosinophilic	**Gastric electrical stimulation (GES) therapy is proven and medically necessary for treating refractory Gastroparesis that has failed other therapies, or chronic intractable (drug-refractory) nausea and vomiting secondary to Gastroparesis of diabetic or idiopathic etiology.** Refer to the *U.S. Food and Drug Administration (FDA)* section of the policy for information regarding FDA labeling and Humanitarian Device Exemption (HDE) for GES.

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Gastrointestinal Motility Disorders, Diagnosis and Treatment (continued)	Aug. 1, 2025	esophagitis (EoE), and nonacid reflux disease (non-GERD), or for the monitoring of treatment response in GERD and EoEI are unproven and not medically necessary due to insufficient evidence of efficacy • Removed language indicating: ○ Rectal manometry, rectal sensation, tone, and compliance test, conventional defecography, and anorectal manometry are proven and medically necessary for evaluation of colorectal function ○ Colonic manometry for evaluating colon motility is unproven and not medically necessary due to insufficient evidence of efficacy ○ Ingestible vibrating capsule devices (e.g., the Vibrant® System) for the treatment of Constipation are unproven and not medically necessary due to insufficient evidence of efficacy **Definitions** • Added definition of "Esophageal Mucosal Integrity Testing" • Removed definition of: ○ Anorectal Manometry ○ Colonic Manometry ○ Defecography **Applicable Codes** • Added CPT codes 43499 and	**The following procedures are unproven and not medically necessary due to insufficient evidence of efficacy:** • Magnetic Resonance Imaging (MRI) Defecography for evaluating Constipation and Anorectal or pelvic floor disorders • Cutaneous, mucous, or serosal Electrogastrography, electroenterography, or body surface gastric mapping (e.g., Gastric Alimetry System, G-Tech Gut Tracker wireless patch system) for diagnosing intestinal or gastric disorders including Gastroparesis • Esophageal Mucosal Integrity Testing by electrical impedance (e.g., MiVu™ Mucosal Integrity Testing System) for the diagnosis of gastroesophageal reflux disease (GERD), eosinophilic esophagitis (EoE) and nonacid reflux disease (non-GERD), or for the monitoring of treatment response in GERD and EoE

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Gastrointestinal Motility Disorders, Diagnosis and Treatment (continued)	Aug. 1, 2025	76498 • Removed CPT/HCPCS codes 74270, 76496, 91117, 91120, 91122, A9286, A9900, A9999, and E1399 **Supporting Information** • Updated *Description of Services*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	
Gynecomastia Surgery	Jul. 1, 2025	**Related Policies *and* Applicable Codes** • Removed reference link to the Medical Policy titled *Panniculectomy and Body Contouring Procedures* **Coverage Rationale** • Replaced language indicating "a mastectomy for treating Gynecomastia *on a* male *member* is considered reconstructive and medically necessary when all the [listed] criteria are met" with "a mastectomy to treat Gynecomastia *in a* male is considered reconstructive and medically necessary when all of the [listed] criteria are met" • Revised coverage criteria for a mastectomy to treat Gynecomastia in a male: ○ Replaced criterion requiring "an appropriate evaluation of medical causes *with* supporting laboratory testing has been normal; *if so*, *lab*	**A mastectomy to treat Gynecomastia in a male under the age of 18 is considered reconstructive and medically necessary when all of the following criteria are met:** • Gynecomastia stage II, III, or IV with moderate to severe chest pain causing a Functional or Physical Impairment (the inability to participate in athletic events, sports, or social activities is not considered to be a Functional or Physical or physiological Impairment) • Glandular breast tissue is the primary cause of Gynecomastia as opposed to fatty deposits (pseudogynecomastia) and is documented on physical exam and/or mammography • Persistent Gynecomastia after cessation of prescribed medications, nutritional supplements, and appropriate screening(s) of non-prescription and/or recreational drugs or substances that have a known side effect of Gynecomastia (examples include but are not limited to testosterone, marijuana, asthma drugs, phenothiazines, anabolic steroids, cimetidine, and calcium channel blockers) • Gynecomastia must be present for at least two years • An appropriate evaluation of medical causes when supporting laboratory testing has been normal; supporting laboratory testing may include but is not limited to the following: ○ Hormone testing (e.g., beta-human chorionic gonadotropin, thyroid function studies, sex-hormone binding globulin, estradiol, follicle-stimulating hormone, luteinizing hormone, prolactin, testosterone) ○ Liver enzymes ○ Serum creatinine ○ Alpha-fetal protein

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Gynecomastia Surgery (continued)	Jul. 1, 2025	*tests might* include but are not limited to the [listed tests], *must be performed*" with "an appropriate evaluation of medical causes *when* supporting laboratory testing has been normal; *supporting laboratory testing may* include but is not limited to the [listed tests]" ○ Revised list of supporting laboratory tests: ▪ Update list of examples of hormone testing; added "thyroid function studies, sex-hormone binding globulin" ▪ Added "alpha-fetal protein" ▪ Removed "thyroid function studies" **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information	**A mastectomy to treat Gynecomastia in a male aged 18 and over is considered reconstructive and medically necessary when all the following criteria are met:** • Gynecomastia stage II, III, or IV with moderate to severe chest pain causing a Functional or Physical Impairment (the inability to participate in athletic events, sports, or social activities is not considered to be a Functional or Physical or physiological Impairment) • Glandular breast tissue is the primary cause of Gynecomastia as opposed to fatty deposits (pseudogynecomastia) and is documented on physical exam and/or mammography • Persistent Gynecomastia after cessation of prescribed medications, nutritional supplements, and appropriate screening(s) of non-prescription and/or recreational drugs or substances that have a known side effect of Gynecomastia (examples include, but are not limited to testosterone, marijuana, asthma drugs, phenothiazines, anabolic steroids, cimetidine, and calcium channel blockers) • An appropriate evaluation of medical causes when supporting laboratory testing has been normal; supporting laboratory testing may include but is not limited to the following: ○ Hormone testing (e.g., beta-human chorionic gonadotropin, thyroid function studies, sex hormone binding globulin, estradiol, follicle-stimulating hormone, luteinizing hormone, prolactin, testosterone) ○ Liver enzymes ○ Serum creatinine ○ Alpha-fetal protein **Note**: Regardless of age, if a tumor or neoplasm is suspected, a breast ultrasound and/or mammogram may be performed with further management as indicated.
Hyperbaric Oxygen Therapy and Topical Oxygen Therapy	Jul. 1, 2025	**Coverage Rationale** • Added language to indicate Mild Hyperbaric Oxygen Therapy (mHBOT) is unproven and not medically necessary for any indication due to insufficient evidence of efficacy; this device does not meet the definition of	**Hyperbaric Oxygen Therapy (HBOT) is medically necessary for the following conditions:** • Acute traumatic peripheral ischemia/insufficiency (i.e., crush injury, reattachment of severed limbs, compartment syndrome) • Air or gas embolism • Anemia, severe, when transfusion is refused, delayed, or unavailable • Avascular necrosis (aseptic osteonecrosis) • Carbon monoxide poisoning

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Hyperbaric Oxygen Therapy and Topical Oxygen Therapy (continued)	Jul. 1, 2025	HBOT • Replaced language indicating "Hyperbaric Oxygen Therapy is unproven and not medically necessary due to insufficient evidence of efficacy for treating and managing all other indications not listed [in the policy] as *proven*" with "Hyperbaric Oxygen Therapy is unproven and not medically necessary due to insufficient evidence of efficacy for treating and managing all other indications not listed [in the policy]as *medically necessary*" **Definitions** • Added definition of "Mild Hyperbaric Oxygen Therapy (mHBOT)" **Supporting Information** • Updated *Description of Services*, *Clinical Evidence*, *FDA*, and *References* sections to reflect the most current information	• Central retinal artery occlusion • Chronic osteomyelitis, refractory to medical and surgical management • Clostridial myonecrosis (gas gangrene) • Compromised skin grafts/flaps • Cyanide poisoning, associated with carbon monoxide poisoning • Decompression sickness • Delayed radiation injuries (soft tissue and bony necrosis) • Diabetic lower extremity wounds • Idiopathic sudden sensorineural hearing loss (ISSHL) • Intracranial abscess • Necrotizing soft tissue infections • Thermal burns, second or third degree **Hyperbaric Oxygen Therapy is unproven and not medically necessary due to insufficient evidence of efficacy for treating and managing all other indications not listed as medically necessary.** **Mild Hyperbaric Oxygen Therapy (mHBOT) is unproven and not medically necessary for any indication due to insufficient evidence of efficacy.** **Note**: This device does not meet the definition of HBOT. **Topical Oxygen Therapy (TOT) is unproven and not medically necessary for the treatment of wounds or ulcers due to insufficient evidence of efficacy.**

Retired

Policy Title	Effective Date	Summary of Changes
Lithotripsy for Salivary Stones	Jun. 1, 2025	• Retired policy; lithotripsy for salivary stones no longer requires clinical review
Unicondylar Spacer Devices for Treatment of Pain or Disability	Jun. 1, 2025	• Retired policy; unicondylar spacer devices for treatment of pain or disability no longer require clinical review

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Amondys 45® (Casimersen)	Jul. 1, 2025	**Coverage Rationale** • Revised coverage criteria for initial therapy; added criterion requiring **one** of the following: ○ The patient has not previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of Duchenne muscular dystrophy (DMD) ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD ▪ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional decline resulting from loss of muscle strength/motor ability (e.g., loss of a motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)]	**Amondys 45 (casimersen) may be covered for the treatment of Duchenne muscular dystrophy (DMD) in patients who meet all of the following criteria:** **Initial Therapy** • Diagnosis of Duchenne muscular dystrophy by, or in consultation with, a neurologist with expertise in the diagnosis of DMD; **and** • Submission of medical records (e.g., chart notes, laboratory values) confirming the mutation of the DMD gene is amenable to exon 45 skipping; **and** • **One** of the following: ○ Submission of medical records (e.g., chart notes, laboratory values) confirming that the patient has a 6-Minute Walk Test (6MWT) ≥ 300 meters while walking independently (e.g., without side-by-side assist, cane, walker, wheelchair, etc.) prior to beginning Amondys 45 therapy; **or** ○ **Both** of the following: ▪ Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** ▪ **One** of the following: – Patient has achieved a score of greater than 17 on the North Star Ambulatory Assessment (NSAA); **or** – Patient has achieved a time to rise (TTR) of less than 7 seconds **and** • **One** of the following: ○ Patient has not previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD; **or** ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD; **and** ▪ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional decline resulting from loss of muscle strength/motor ability (e.g., loss of a

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Amondys 45® (Casimersen) (continued)	Jul. 1, 2025		motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] **and** • Amondys 45 is not used concomitantly with other exon skipping therapies for DMD [e.g., Exondys 51 (eteplirsen), Viltepso (viltolarsen), Vyondys 53 (golodirsen)]; **and** • Prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** • Initial authorization will be for no more than 12 months **Continuation Therapy** • Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** • Amondys 45 is not used concomitantly with other exon skipping therapies for DMD [e.g., Exondys 51 (eteplirsen), Viltepso (viltolarsen), Vyondys 53 (golodirsen)]; **and** • Prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** • Reauthorization will be for no more than 12 months **Unproven** **Amondys 45 is unproven and not medically necessary for the treatment of other forms of muscular dystrophy (e.g., Becker muscular dystrophy).**
Clotting Factors, Coagulant Blood Products, & Other Hemostatics	Jul. 1, 2025	**Coverage Rationale** • Revised list of applicable products and corresponding brand names: ○ Added: ▪ Antithrombin-directed small interfering	Refer to the policy for complete details.

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Clotting Factors, Coagulant Blood Products, & Other Hemostatics (continued)	Jul. 1, 2025	ribonucleic acid (siRNA) [brand name Qfitlia® (fitusiran)] ▪ Tissue factor pathway inhibitor (TFPI) antagonist [brand names Alhemo® (concizumab-mtci) and Hympavzi™ (marstacimab-hncq)] ○ Updated list of brand names for: ▪ Factor VIII (recombinant); removed Helixate® FS [antihemophilic factor (recombinant)] ▪ Factor IX (plasma-derived); removed Mononine® [coagulation factor IX (human)] • Added language to indicate: ○ **Alhemo (concizumab-mtci)** and **Qfitlia (fitusiran)** have been added to the Review at Launch program and some members may not be eligible for coverage of these medications at this time; refer to the Medical Benefit Drug Policy titled *Review at Launch for New to Market Medications* for additional details ○ **Concizumab-mtci [Alhemo]** is proven/medically necessary for routine prophylaxis to prevent or reduce the frequency of	

Revised			
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Clotting Factors, Coagulant Blood Products, & Other Hemostatics (continued)	Jul. 1, 2025	bleeding episodes in patients with hemophilia A with factor VIII inhibitors/ hemophilia B with factor IX inhibitors when all of the criteria [listed in the policy] are met (note that concizumab-mtci [Alhemo] is a self-injectable medication that should be obtained under the member's pharmacy benefit unless the criteria [listed in the policy] are met) ○ **Fitusiran [Qfitlia]** is proven/medically necessary for routine prophylaxis to prevent or reduce the frequency of bleeding episodes in patients with hemophilia A/hemophilia B when all of the criteria [listed in the policy] are met (note that fitusiran [Qfitlia] is a self-injectable medication that should be obtained under the member's pharmacy benefit unless the criteria [listed in the policy] are met) ○ **Marstacimab-hncq [Hympavzi]** is proven/medically necessary for routine prophylaxis to prevent or reduce the frequency of bleeding episodes in patients with hemophilia B without factor IX inhibitors when all of the criteria [listed in the policy]	

Revised			
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Clotting Factors, Coagulant Blood Products, & Other Hemostatics (continued)	Jul. 1, 2025	are met (note that marstacimab-hncq [Hympavzi] is a self-injectable medication that should be obtained under the member's pharmacy benefit unless the criteria [listed in the policy] are met) ○ **Marstacimab-hncq [Hympavzi]** is not medically necessary for routine prophylaxis to prevent or reduce the frequency of bleeding episodes in patients with hemophilia A without factor VIII inhibitors • Removed content addressing: ○ Helixate® FS [antihemophilic factor (recombinant)] ○ Mononine® [coagulation factor IX (human)] • Revised coverage criteria for **emicizumab-kxwh [Hemlibra]**; removed criterion requiring prescriber attestation that the patient is not to receive extended half-life factor VIII replacement products (e.g., Adynovate, Afstyla, Altuviiio, Eloctate, Jivi) for the treatment of breakthrough bleeding episodes **Applicable Codes** • Added HCPCS codes C9399, J3490, J3590, and J7172 **Supporting Information** • Updated *Background*, *Benefit Considerations*, *Clinical*	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Clotting Factors, Coagulant Blood Products, & Other Hemostatics (continued)	Jul. 1, 2025	*Evidence*, *FDA*, and *References* sections to reflect the most current information	
Entyvio® (Vedolizumab)	Jul. 1, 2025	**Coverage Rationale** • Added language to indicate Entyvio (vedolizumab) is proven and medically necessary for the treatment of gastrointestinal (GI) acute graft-versus-host disease (aGVHD) when all of the following criteria are met: ***Initial Therapy*** ○ Diagnosis of steroid-refractory acute GI aGVHD ○ One of the following: ▪ Patient is receiving Entyvio in combination with systemic corticosteroids ▪ Patient is intolerant to systemic corticosteroid therapy ○ Initial authorization is for no more than 4 doses ***Continuation of Therapy*** ○ Documentation of positive clinical response ○ Patient continues to experience acute GI aGVHD ○ One of the following: ▪ Patient is receiving Entyvio in combination with systemic corticosteroids ▪ Patient is intolerant to	**This policy refers to Entyvio (vedolizumab) injection for intravenous infusion.** **Entyvio (vedolizumab) is proven and medically necessary for the treatment of:** • **Crohn's disease** when **all** of the following criteria are met: ○ For **initial therapy**, **all** of the following: ▪ Diagnosis of moderately to severely active Crohn's disease (CD); **and** ▪ **One** of the following: – History of failure, contraindication, or intolerance to at least **one** of the following conventional therapies: • Tumor necrosis factor (TNF) blocker [e.g., adalimumab, Cimzia (certolizumab)] • Immunomodulator (e.g., azathioprine, 6-mercaptopurine) • Corticosteroid – Corticosteroid dependent (e.g., unable to successfully taper corticosteroids without a return of the symptoms of CD); **and** ▪ Entyvio is initiated and titrated according to US Food and Drug Administration (FDA) labeled dosing for Crohn's disease; **and** ▪ Patient is **not** receiving Entyvio in combination with either of the following: – Targeted immunomodulator [e.g., adalimumab, Cimzia (certolizumab), Omvoh (mirikizumab-mrkz), Rinvoq (upadacitinib), Simponi (golimumab), Skyrizi (risankizumab), Tremfya (guselkumab), ustekinumab, Xeljanz (tofacitinib)] – Tysabri (natalizumab) **and** ▪ Initial authorization will be for no more than 12 months ○ For **continuation of therapy**, **all** of the following: ▪ Documentation of positive clinical response to Entyvio; **and** ▪ Entyvio dosing for Crohn's disease is in accordance with the

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Entyvio® (Vedolizumab) (continued)	Jul. 1, 2025	systemic corticosteroid therapy ○ Authorization is for no more than 4 doses • Revised coverage criteria for **immune checkpoint inhibitor-related toxicities**; added criterion to allow coverage for one of the following immunotherapy-related diagnoses: ○ Moderate to severe esophagitis, gastritis, or duodenitis if no improvement on corticosteroids or budesonide ○ Mild (G1) diarrhea or colitis if persistent or progressive symptoms and positive lactoferrin/calprotectin • Updated list of examples of targeted immunomodulators the patient must not be receiving in combination with Entyvio: ***Crohn's Disease*** ○ Added: ▪ Omvoh (mirikizumab-mrkz) ▪ Simponi (golimumab) ▪ Skyrizi (risankizumab) ▪ Tremfya (guselkumab) ○ Removed: ▪ Infliximab ▪ Xeljanz XR ○ Replaced "*Stelara* (ustekinumab)" with "ustekinumab"	FDA labeled dosing; **and** ▪ Reauthorization will be for no more than 12 months • **Ulcerative colitis** when **all** of the following criteria are met: ○ For **initial therapy**, **all** of the following: ▪ Diagnosis of moderately to severely active ulcerative colitis (UC); **and** ▪ **One** of the following: – History of failure, contraindication, or intolerance to at least **one** of the following conventional therapies: • Tumor necrosis factor (TNF) blocker [e.g., adalimumab, Simponi (golimumab)] • Immunomodulator (e.g., azathioprine, 6-mercaptopurine) • Corticosteroid – Corticosteroid dependent (e.g., unable to successfully taper corticosteroids without a return of the symptoms of UC) **and** ▪ Entyvio is initiated and titrated according to FDA labeled dosing for ulcerative colitis; **and** ▪ Patient is **not** receiving Entyvio in combination with either of the following: – Targeted immunomodulator [e.g., adalimumab, Cimzia (certolizumab), Enbrel (etanercept), Omvoh (mirikizumab-mrkz), Rinvoq (upadacitinib), Simponi (golimumab), Skyrizi (risankizumab), Tremfya (guselkumab), ustekinumab, Xeljanz (tofacitinib)] – Tysabri (natalizumab) **and** ▪ Initial authorization will be for no more than 12 months ○ For **continuation of therapy**, **all** of the following: ▪ Documentation of positive clinical response to Entyvio; **and** ▪ Entyvio dosing for ulcerative colitis is in accordance with the FDA labeled dosing; **and** ▪ Reauthorization will be for no more than 12 months • **Immune checkpoint inhibitor-related toxicities** when all of the following criteria are met for **initial and continuation of therapy**: ○ **One** of the following immunotherapy-related diagnosis: ▪ Moderate to severe esophagitis, gastritis, or duodenitis if no

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Entyvio® (Vedolizumab) (continued)	Jul. 1, 2025	***Ulcerative Colitis*** ○ Added: ▪ Cimzia (certolizumab) ▪ Enbrel (etanercept) ▪ Omvoh (mirikizumab-mrkz) ▪ Skyrizi (risankizumab) ▪ Tremfya (guselkumab) ○ Removed: ▪ Infliximab ▪ Xeljanz XR ○ Replaced "*Stelara* (ustekinumab)" with "ustekinumab" **Applicable Codes** • Added ICD-10 diagnosis code D89.810 **Supporting Information** • Updated *FDA* and *References* sections to reflect the most current information	improvement on corticosteroids or budesonide ▪ Mild (G1) diarrhea or colitis if persistent or progressive symptoms and positive lactoferrin/calprotectin ▪ Moderate (G2) or severe (G3-4) immunotherapy-related diarrhea or colitis **and** ○ Patient is receiving a checkpoint inhibitor [e.g., Keytruda (Pembrolizumab), Opdivo (Nivolumab)]; **and** ○ **One** of the following: ▪ History of failure, contraindication, or intolerance to infliximab ▪ Patient has immune-related hepatitis **and** ○ Authorization will be for no more than 3 doses of Entyvio • **GI acute graft-versus-host disease (aGVHD)** when all of the following criteria are met: ○ For **initial therapy**, **all** of the following: ▪ Diagnosis of steroid-refractory acute GI aGVHD; **and** ▪ **One** of the following: – Patient is receiving Entyvio in combination with systemic corticosteroids – Patient is intolerant to systemic corticosteroid therapy **and** ▪ Initial authorization is for no more than 4 doses ○ For **continuation of therapy**, **all** of the following: ▪ Documentation of positive clinical response; **and** ▪ Patient continues to experience acute GI aGVHD; **and** ▪ **One** of the following: – Patient is receiving Entyvio in combination with systemic corticosteroids – Patient is intolerant to systemic corticosteroid therapy **and** ▪ Authorization is for no more than 4 doses
Exondys 51® (Eteplirsen)	Jul. 1, 2025	**Coverage Rationale** • Revised coverage criteria for initial therapy; added criterion requiring **one** of the following: ○ The patient has not	**Exondys 51® (eteplirsen) may be covered for the treatment of Duchenne muscular dystrophy (DMD) in patients who meet all of the following criteria:** **Initial Therapy**

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Exondys 51® (Eteplirsen) (continued)	Jul. 1, 2025	previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of Duchenne muscular dystrophy (DMD) ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD ▪ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional decline resulting from loss of muscle strength/motor ability (e.g., loss of a motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)]	• Diagnosis of Duchenne muscular dystrophy by, or in consultation with, a neurologist with expertise in the diagnosis of DMD; **and** • Submission of medical records (e.g., chart notes, laboratory values) confirming the mutation of the DMD gene is amenable to exon 51 skipping; **and** • **One** of the following: ○ Submission of medical records (e.g., chart notes, laboratory values) confirming that the patient has a 6-Minute Walk Test (6MWT) ≥ 300 meters while walking independently (e.g., without side-by-side assist, cane, walker, wheelchair, etc.) prior to beginning Exondys 51 therapy; **or** ○ **Both** of the following: ▪ Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** ▪ **One** of the following: – Patient has achieved a score of greater than 17 on the North Star Ambulatory Assessment (NSAA); **or** – Patient has achieved a time to rise (TTR) of less than 7 seconds **and** • **One** of the following: ○ Patient has not previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD; **or** ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD; **and** ▪ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional decline resulting from loss of muscle strength/motor ability (e.g., loss of a motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] **and** • Exondys 51 is not used concomitantly with other exon skipping therapies for DMD [e.g., Amondys (casimersen), Viltepso (viltolarsen), Vyondys 53

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Exondys 51® (Eteplirsen) (continued)	Jul. 1, 2025		(golodirsen)]; **and** • Exondys 51 is prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** • Initial authorization will be for no more than 12 months **Continuation Therapy** • Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** • Exondys 51 is not used concomitantly with other exon skipping therapies for DMD [e.g., Amondys (casimersen), Viltepso (viltolarsen), Vyondys 53 (golodirsen)]; **and** • Prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** • Reauthorization will be for no more than 12 months **Unproven** **Exondys 51 is unproven and not medically necessary for the treatment of other forms of muscular dystrophy (e.g., Becker muscular dystrophy).**
Korsuva® (Difelikefalin)	Jul. 1, 2025	**Coverage Rationale** • Revised proven coverage criteria; added criteria for continuation of therapy requiring: ○ Documentation of a positive clinical response (i.e., reduction in itch from baseline) ○ Dosing is in accordance with the U.S. FDA approved labeling ○ Reauthorization will be for no	**Korsuva (difelikefalin) is proven for the treatment of moderate-to-severe pruritus associated with chronic kidney disease in adults undergoing hemodialysis when the following criteria are met:** • For **initial therapy**, **all** of the following: ○ Diagnosis of moderate-to-severe pruritus associated with chronic kidney disease; **and** ○ Patient is on hemodialysis; **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Initial authorization will be for no longer than 12 months • For **continuation of therapy**, **all** of the following: ○ Documentation of a positive clinical response (i.e., reduction in itch

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Korsuva® (Difelikefalin) (continued)	Jul. 1, 2025	longer than 12 months **Supporting Information** • Updated *Background* and *References* sections to reflect the most current information	from baseline); **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Reauthorization will be for no longer than 12 months **Korsuva (difelikefalin) is medically necessary for the treatment of moderate-to-severe pruritus associated with chronic kidney disease in adults undergoing hemodialysis when the following criteria are met:** • For **initial therapy**, **all** of the following: ○ Diagnosis of moderate-to-severe pruritus associated with chronic kidney disease; **and** ○ Patient is on hemodialysis; **and** ○ Pruritus is not attributed to a cause other than end stage renal disease or its complications (e.g., pruritic dermatological disease, cholestatic liver disease); **and** ○ Pruritus is not limited to occurring only during the dialysis session; **and** ○ Pruritis is not localized to just the palms of the hands; **and** ○ History of failure, contraindication, or intolerance to other pruritis treatments (e.g., antihistamines, corticosteroids, gabapentin, pregabalin, capsaicin); **and** ○ Prescribed by or in consultation with a nephrologist; **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Initial authorization will be for no longer than 12 months • For **continuation of therapy**, **all** of the following: ○ Documentation of a positive clinical response (i.e., reduction in itch from baseline); **and** ○ Prescribed by or in consultation with a nephrologist; **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Reauthorization will be for no longer than 12 months
Maximum Dosage and Frequency	Jul. 1, 2025	**Related Policies** • Added reference link to the Medical Benefit Drug Policy titled *Niktimvo™ (Axatilimab-Csfr)* **Coverage Rationale**	This policy provides information about the maximum dosage per administration and dosing frequency for certain medications administered by a medical professional. Most medications have a maximum dosage and frequency based upon body surface area or patient weight or a set maximal dosage and frequency independent of patient body size.

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Maximum Dosage and Frequency (continued)	Jul. 1, 2025	• Revised list of applicable drug products; added: ○ aflibercept-ayyh (Pavblu™) ○ axatilimab-csfr (Niktimvo™) ○ tocilizumab-aazg (Tyenne®) ○ tocilizumab-bavi (Tofidence™) ○ ustekinumab-aauz (Otulfi™) ○ ustekinumab-aekn (Selarsdi™) ○ ustekinumab-auub (Wezlana™) ○ ustekinumab-ttwe (Pyzchiva®) ***Maximum Allowed Quantities by HCPCS Units*** • Added maximum allowed quantities for: ○ Niktimvo (axatilimab-csfr) (HCPCS code J9038) ○ Otulfi (ustekinumab-aauz) (HCPCS code Q9999) ○ Pyzchiva (ustekinumab-ttwe) (HCPCS codes Q9996 and Q9997) ○ Selarsdi (ustekinumab-aekn) (HCPCS code Q9998) ○ Tofidence (tocilizumab-bavi) (HCPCS code Q5133) ○ Tyenne (tocilizumab-aazg) (HCPCS code Q5135) ○ Wezlana (ustekinumab-auub) (HCPCS codes Q5137 and Q5138) ***Maximum Allowed Quantities for National Drug Code (NDC) Billing*** • Added maximum allowed quantities for:	**Drug Products** • abatacept (Orencia®) • abobotulinumtoxinA (Dysport®) • aflibercept (Eylea®) • aflibercept (Eylea® HD) • aflibercept-ayyh (Pavblu™) • atezolizumab (Tecentriq®) • avelumab (Bavencio®) • axatilimab-csfr (Niktimvo™) • benralizumab (Fasenra®) • bevacizumab (Avastin®) • bevacizumab-adcd (Vegzelma®) • bevacizumab-awwb (Mvasi™) • bevacizumab-bvzr (Zirabev®) • bevacizumab-maly (Alymsys®) • brolucizumab-dbll (Beovu®) • canakinumab (Ilaris®) • cemiplimab-rwlc (Libtayo®) • certolizumab pegol (Cimzia®) • crovalimab-akkz (PiaSky™) • daxibotulinumtoxina-lanm (Daxxify®) • denosumab (Prolia® & Xgeva®) • durvalumab (Imfinzi®) • eculizumab (Soliris®) • edaravone (Radicava®) • efgartigimod alfa-fcab (Vyvgart®) • efgartigimod alfa and hyaluronidase-qvfc (Vyvgart® Hytrulo) • pegfilgrastim-cbqv (Udenyca®) • pegfilgrastim-fpgk (Stimufend®) • pegfilgrastim-jmdb (Fulphila™) • pegfilgrastim-pbbk (Fylnetra®) • pegfilgrastim-bmez (Ziextenzo®) • pegloticase (Krystexxa®) • pembrolizumab (Keytruda®) • ranibizumab (Lucentis®) • ranibizumab-nuna (Byooviz™) • ranibizumab-eqrn (Cimerli™) • ravulizumab-cwvz (Ultomiris®) • reslizumab (Cinqair®) • rimabotulinumtoxinB (Myobloc®) • risankizumab-rzaa (Skyrizi®) • rituximab (Rituxan®) • rituximab-pvvr (Ruxience™) • rituximab-abbs (Truxima®) • rituximab-arrx (Riabni™) • rituximab and hyaluronidase (Rituxan Hycela®) • rozanolixizumab-noli (Rystiggo®) • spesolimab-sbzo (Spevigo®) • testosterone cypionate (Depo-Testosterone®) • testosterone enanthate • testosterone pellets (Testopel®) • testosterone undecanoate (Aveed®)

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Maximum Dosage and Frequency (continued)	Jul. 1, 2025	○ Niktimvo (axatilimab-csfr) (NDCs 50881-0012-10, 50881-0023-11, and 50881-0034-12) ○ Otulfi (ustekinumab-aauz) (NDCs 65219-0824-01, 65219-0826-26, and 65219-0828-05) ○ Pyzchiva (ustekinumab-ttwe) (NDCs 61314-0651-01, 61314-0652-01, and 61314-0654-94) ○ Selarsdi (ustekinumab-aekn) (NDCs 51759-0505-32, 51759-0607-32, and 51759-0708-13) ○ Tofidence (tocilizumab-bavi) (NDCs 64406-0022-01, 64406-0023-01, and 64406-0024-01) ○ Tyenne (tocilizumab-aazg) (NDCs 65219-0590-04, 65219-0592-10, and 65219-0594-20) ○ Wezlana (ustekinumab-auub) (NDCs 84612-0055-01, 84612-0066-01, 84612-0076-01, 84612-0089-01, 84612-0855-01, 84612-0876-01, and 84612-0889-01) ***Maximum Allowed Frequencies*** • Added maximum allowed frequencies for: ○ Niktimvo (axatilimab-csfr) ○ Otulfi (ustekinumab-aauz) ○ Pavblu (aflibercept-ayyh) ○ Pyzchiva (ustekinumab-ttwe)	• eflapegrastim-xnst (Rolvedon™) • emicizumab-kxwh (Hemlibra®) • eptinezumab-jjmr (Vyepti®) • faricimab-svoa (Vabysmo™) • golimumab (Simponi Aria®) • guselkumab (Tremfya®) • inclisiran (Leqvio®) • incobotulinumtoxinA (Xeomin®) • infliximab (Remicade®) • infliximab-axxq (Avsola™) • infliximab-dyyb (Inflectra®) • infliximab-abda (Renflexis®) • ipilimumab (Yervoy®) • mepolizumab (Nucala®) • mirikizumab-mrkz (Omvoh®) • nivolumab (Opdivo®) • ocrelizumab (Ocrevus®) • omalizumab (Xolair®) • onabotulinumtoxinA (Botox®) • patisiran (Onpattro®) • pegcetacoplan (Syfovre™) • pegfilgrastim (Neulasta®) • pegfilgrastim-apgf (Nyvepria™) • tezepelumab-ekko (Tezspire®) • tildrakizumab-asmn (Ilumya™) • tocilizumab (Actemra®) • tocilizumab-aazg (Tyenne®) • tocilizumab-bavi (Tofidence™) • tofersen (Qalsody™) • trastuzumab (Herceptin®) • trastuzumab-anns (Kanjinti™) • trastuzumab-dkst (Ogivri™) • trastuzumab-dttb (Ontruzant®) • trastuzumab-pkrb (Herzuma®) • trastuzumab-qyyp (Trazimera™) • ustekinumab (Stelara®) • ustekinumab-aauz (Otulfi™) • ustekinumab-aekn (Selarsdi) • ustekinumab-auub (Wezlana™) • ustekinumab-ttwe (Pyzchiva®) • vedolizumab (Entyvio®) • vutrisiran (Amvuttra™) • zoledronic acid (zoledronic acid, Reclast®) **The use of medications included in this policy when given within the maximum dosage and/or frequency based upon body surface area or patient weight or a set of maximal dosage and/or frequency independent of patient body size are proven when used according to labeled indications or when otherwise supported by published clinical evidence [e.g., well-designed systematic reviews (with or without meta-analyses) of multiple well-designed randomized controlled trials, the National Comprehensive Cancer Network (NCCN) guidelines].**

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Maximum Dosage and Frequency (continued)	Jul. 1, 2025	○ Selarsdi (ustekinumab-aekn) ○ Tofidence (tocilizumab-bavi) ○ Tyenne (tocilizumab-aazg) ○ Wezlana (ustekinumab-auub) **Applicable Codes** • Added HCPCS codes J9038, Q5133, Q5135, Q5137, Q5138, Q5147, Q9996, Q9997, Q9998, and Q9999 • Added NDCs 50881-0012-10, 50881-0023-11, 50881-0034-12, 51759-0505-32, 51759-0607-32, 51759-0708-13, 61314-0651-01, 61314-0652-01, 61314-0654-94, 64406-0022-01, 64406-0023-01, 64406-0024-01, 65219-0590-04, 65219-0592-10, 65219-0594-20, 65219-0824-01, 65219-0826-26, 65219-0828-05, 84612-0055-01, 84612-0066-01, 84612-0076-01, 84612-0089-01, 84612-0855-01, 84612-0876-01, and 84612-0889-01 **Supporting Information** • Updated *References* section to reflect the most current information	**The use of medications included in this policy when given beyond maximum dosages and/or frequency based upon body surface area or patient weight or a set maximal dosage independent of patient body size are not supported by package labeling or published clinical evidence and are unproven.** **Continued use of a medication or dosages used beyond labeled indication or other published clinical evidence [e.g., well-designed systematic reviews (with or without meta-analyses) of multiple well-designed randomized controlled trials, NCCN guidelines] is considered not medically necessary.** This policy creates an upper dose limit based on the clinical evidence and the 95th percentile for adult body weight (140 kg) and body surface area (2.71 meters^2) in the U.S. (adult male, 30 to 39 years, Fryar, 2021). In some cases, the maximum allowed units and/or vials may exceed the upper level limit as defined within this policy due to an individual patient body weight > 140 kg or body surface area > 2.71 meters^2. Refer to the policy for complete details.
Oncology Medication Clinical Coverage	Jul. 1, 2025	**Coverage Rationale** • Revised list of UnitedHealthcare preferred and non-preferred oncology products; added: ○ Bortezomib (HCPCS codes J9041, J9049, and J9051) (preferred for all oncology indications) ○ Boruzu (HCPCS code J9054) (non-preferred for all	**Description** This policy provides parameters for coverage of injectable oncology medications (including, but not limited to octreotide acetate, leuprolide acetate, leucovorin, and levoleucovorin), including therapeutic radiopharmaceuticals, covered under the medical benefit based upon the National Comprehensive Cancer Network (NCCN) Drugs & Biologics Compendium® (NCCN Compendium®). The Compendium lists the appropriate drugs and biologics for specific cancers using US Food and Drug Administration (FDA)-approved disease indications and specific NCCN panel recommendations. Each recommendation is supported by a level of evidence

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Oncology Medication Clinical Coverage (continued)	Jul. 1, 2025	oncology indications) ○ Cyclophosphamide (HCPCS codes J9071, J9073, and J9075) (preferred for all oncology indications) ○ Cyclophosphamide (HCPCS codes J9074 and J9076) (non-preferred for all oncology indications) ○ Docetaxel (HCPCS code J9171) (preferred for all oncology indications) ○ Docivyx (HCPCS code J9172) (non-preferred for all oncology indications) ○ Frindovyx (HCPCS code J9072) (non-preferred for all oncology indications) ○ Opdivo Qvantig (nivolumab and hyaluronidase-nvhy) (non-preferred for head and neck cancers: recurrent, unresectable, oligometastatic, or metastatic disease, nasopharyngeal) ○ Tecentriq Hybreza (atezolizumab and hyaluronidase-tqjs) (preferred for non-small cell lung cancer: advanced or metastatic, monotherapy, PD-L1 expression positive ≥ 50%) ○ Tevimbra (tislelizumab-jsgr) (non-preferred for head and neck cancers: recurrent, unresectable, oligometastatic, or metastatic disease,	category. Coverage of White Blood Cell Colony Stimulating Factors and Erythropoiesis-Stimulating Agents are addressed in separate policies. This policy does not provide coverage criteria for Chimeric Antigen Receptor (CAR)-T cell or Tumor-Infiltrating Lymphocyte (TIL) cell products. Coverage determinations are based on the member's benefits and the OptumHealth Transplant Solutions criteria for covered transplants; refer to the Clinical Guideline titled Chimeric Antigen Receptor T-cell (CAR T) Therapy, Tumor-Infiltrating Lymphocyte (TIL) Cell Therapy. **Coverage Rationale** ***Medical Necessity Plans*** The Oncology Products table [in the policy] lists the UnitedHealthcare preferred oncology products and respective non-preferred products. Coverage will be provided for the UnitedHealthcare preferred oncology product contingent on the coverage criteria in the *Diagnosis-Specific Criteria* section. Coverage for any respective non-preferred oncology product will be provided contingent on the criteria in the *Preferred Product Criteria* and the *Diagnosis-Specific Criteria* sections. Members new to therapy will be required to utilize the UnitedHealthcare preferred oncology product unless they meet the criteria in this section. ***Preferred Product Criteria*** ***(For Medicare reviews, refer to the CMS section of the policy.)*** **Treatment with the respective non-preferred product specified in the Oncology Products table [in the policy] is medically necessary for oncology indications when both of the following are met:** • History of intolerance or contraindication to one of the UnitedHealthcare's preferred oncology products; and • Physician attests that, in their clinical opinion, the same intolerance, contraindication, or adverse event would not be expected to occur with the respective non-preferred product **Oncology Products** Refer to the policy for a list of UnitedHealthcare preferred and non-preferred

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Oncology Medication Clinical Coverage (continued)	Jul. 1, 2025	nasopharyngeal) **Applicable Codes** • Added HCPCS codes J9024, J9041, J9049, J9051, J9054, J9071, J9072, J9073, J9074, J9075, J9076, J9171, J9172, and J9329 **Supporting Information** • Updated *CMS* section to reflect the most current information	oncology products and corresponding indications. Any U.S. Food and Drug Administration approved product that may belong to UnitedHealthcare Preferred or Non-preferred Oncology Product categories but not listed by name in this policy will be considered non-preferred until reviewed by UnitedHealthcare P&T committee. **Diagnosis-Specific Criteria** ***Injectable Oncology Medications*** UnitedHealthcare recognizes indications and uses of injectable oncology medications, including therapeutic radiopharmaceuticals, listed in the NCCN Drugs and Biologics Compendium with Categories of Evidence and Consensus of 1, 2A, and 2B as proven and medically necessary, and Categories of Evidence and Consensus of 3 as unproven and not medically necessary. UnitedHealthcare will cover all chemotherapy agents for individuals under the age of 19 years for oncology indications. The majority of pediatric patients receive treatments on national pediatric protocols that are quite similar in concept to the NCCN patient care guidelines. Refer to *Preferred Product Criteria* for the UnitedHealthcare preferred oncology products and indications.

Oxlumo® (Lumasiran) and Rivfloza® (Nedosiran)	Jul. 1, 2025	**Coverage Rationale**
		• Revised coverage criteria for initial therapy for Rivfloza; replaced criterion requiring "the patient is at least ***9*** years of age and older" with "the patient is at least ***2*** years of age and older"
		Supporting Information
		• Updated *FDA* and *References* sections to reflect the most current information
			This policy provides information about the use of certain specialty pharmacy medications administered by either the subcutaneous (SC) or intravenous (IV) route by a healthcare professional (HCP).
			
			Oxlumo is proven for the treatment of primary hyperoxaluria type 1 (PH1) in patients who meet all of the following criteria:
			• For **initial therapy**, **all** of the following:
			  ○ Diagnosis of PH1; **and**
			  ○ Patient is not receiving Oxlumo in combination with Rivfloza (nedosiran); **and**
			  ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and**
			  ○ Initial authorization will be for no more than 12 months
			• For **continuation of therapy**, **all** of the following:

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Oxlumo® (Lumasiran) and Rivfloza® (Nedosiran) (continued)	Jul. 1, 2025		○ Documentation of positive clinical response to Oxlumo; **and** ○ Patient is not receiving Oxlumo in combination with Rivfloza (nedosiran); **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Reauthorization will be for no more than 12 months **Oxlumo is medically necessary for the treatment of PH1 in patients who meet all of the following criteria:** • For **initial therapy**, **all** of the following: ○ Diagnosis of PH1 by, or in consultation with, a specialist (e.g., geneticist, nephrologist, urologist) with expertise in the diagnosis of PH1; **and** ○ Confirmation of the PH1 diagnosis based on **both** of the following: ▪ Metabolic testing demonstrating **one** of the following: – Increased urinary oxalate excretion [e.g., greater than 1 mmol/1.73 m2 per day (90 mg/1.73 m2 per day), increased urinary oxalate: creatinine ratio relative to normative values for age]; **or** – Increased plasma oxalate and glyoxylate concentrations **and** ▪ Genetic testing has confirmed a mutation in the alanine: glyoxylate aminotransferase (AGT or AGXT) gene **and** ○ Patient has not received a liver transplant; **and** ○ Prescribed by, or in consultation with, a specialist (e.g., geneticist, nephrologist, urologist) with expertise in the treatment of PH1; **and** ○ Patient is not receiving Oxlumo in combination with Rivfloza (nedosiran); **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Initial authorization will be for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Submission of medical records (e.g., chart notes, laboratory values) documenting a positive clinical response to therapy from pre-treatment baseline (e.g., decreased urinary oxalate concentrations, decreased urinary oxalate: creatinine ratio, decreased plasma

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Oxlumo® (Lumasiran) and Rivfloza® (Nedosiran) (continued)	Jul. 1, 2025		oxalate concentrations); **and** ○ Patient has not received a liver transplant; **and** ○ Patient is not receiving Oxlumo in combination with Rivfloza (nedosiran); **and** ○ Prescribed by, or in consultation with, a specialist (e.g., geneticist, nephrologist, urologist) with expertise in the treatment of PH1; **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Reauthorization will be for no more than 12 months **Rivfloza for provider administration is proven for the treatment of primary hyperoxaluria type 1 (PH1) in patients who meet all of the following criteria:** • For **initial therapy**, **all** of the following: ○ Diagnosis of PH1; **and** ○ Patient is at least 2 years of age and older; **and** ○ Patient has relatively preserved kidney function (e.g., eGFR ≥ 30 mL/min/1.73 m^2); **and** ○ Patient is not receiving Rivfloza in combination with Oxlumo; **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Initial authorization will be for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Documentation of positive clinical response to Rivfloza; **and** ○ Patient has relatively preserved kidney function (e.g., eGFR ≥ 30 mL/min/1.73 m^2); **and** ○ Patient is not receiving Rivfloza in combination with Oxlumo (lumasiran); **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Reauthorization will be for no more than 12 months **Rivfloza for provider administration is medically necessary for the treatment of PH1 in patients who meet all of the following criteria:** • For **initial therapy**, **all** of the following: ○ Diagnosis of PH1 by, or in consultation with, a specialist (e.g., geneticist, nephrologist, urologist) with expertise in the diagnosis of

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Oxlumo® (Lumasiran) and Rivfloza® (Nedosiran) (continued)	Jul. 1, 2025		PH1; **and** ○ Confirmation of the PH1 diagnosis based on **both** of the following: ▪ Metabolic testing demonstrating **one** of the following: – Increased urinary oxalate excretion [e.g., greater than 1 mmol/1.73 m2 per day (90 mg/1.73 m2 per day), increased urinary oxalate: creatinine ratio relative to normative values for age]; or – Increased plasma oxalate and glyoxylate concentrations **and** ▪ Genetic testing has confirmed a mutation in the alanine: glyoxylate aminotransferase (AGT or AGXT) gene **and** ○ Patient has not received a liver transplant; **and** ○ Patient is at least 2 years of age and older; **and** ○ Patient has relatively preserved kidney function (e.g., eGFR ≥ 30 mL/min/1.73 m^2); **and** ○ Patient is not receiving Rivfloza in combination with Oxlumo (lumasiran); **and** ○ Prescribed by, or in consultation with, a specialist (e.g., geneticist, nephrologist, urologist) with expertise in the treatment of PH1; **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Initial authorization will be for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Submission of medical records (e.g., chart notes, laboratory values) documenting a positive clinical response to therapy from pre-treatment baseline (e.g., decreased urinary oxalate concentrations, decreased urinary oxalate: creatinine ratio, decreased plasma oxalate concentrations); **and** ○ Patient has not received a liver transplant; **and** ○ Patient has relatively preserved kidney function (e.g., eGFR ≥ 30 mL/min/1.73 m^2); **and** ○ Patient is not receiving Rivfloza in combination with Oxlumo (lumasiran); **and** ○ Prescribed by, or in consultation with, a specialist (e.g., geneticist, nephrologist, urologist) with expertise in the treatment of PH1; **and** ○ Dosing is in accordance with the United States Food and Drug

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Oxlumo® (Lumasiran) and Rivfloza® (Nedosiran) (continued)	Jul. 1, 2025		Administration approved labeling; **and** ○ Reauthorization will be for no more than 12 months
Spevigo® (Spesolimab-Sbzo)	Jul. 1, 2025	**Coverage Rationale** • Revised coverage criteria: ○ Replaced criterion requiring: ▪ "Presence of primary, sterile, macroscopically visible pustules on *non-acral skin*" with "presence of primary, sterile, macroscopically visible pustules on *erythematous base*" ▪ "Pustulation is not restricted to the psoriatic plaques" with "pustulation is not restricted to the *acral region or within* psoriatic plaques" ▪ "The patient is not receiving Spevigo in combination with another targeted immunomodulator" with "the patient is not receiving Spevigo in combination with another targeted immunomodulator *for treatment of the same indication*" ○ Added criterion for continuation of **subcutaneous** Spevigo therapy requiring reduction in	**Generalized Pustular Psoriasis (GPP)** **Spevigo for intravenous use is proven for the treatment of generalized pustular psoriasis flares when all of the following criteria are met:** • Diagnosis of generalized pustular psoriasis (GPP); **and** • Patient has a GPP flare; **and** • Spevigo is dosed according to U.S. Food and Drug Administration labeled dosing for GPP flares; **and** • Total dose of Spevigo does not exceed two doses per single GPP flare [**Note**: If the patient has been treated with Spevigo for a previous GPP flare, then a new (different) GPP flare may be treated with up to two doses of Spevigo.]; **and** • Authorization will be for no more than 21 days **Spevigo for intravenous use is medically necessary for the treatment of generalized pustular psoriasis flares when all of the following criteria are met:** • Diagnosis of generalized pustular psoriasis (GPP) based on **both** of the following: ○ Presence of primary, sterile, macroscopically visible pustules on erythematous base; **and** ○ Pustulation is not restricted to the acral region or within psoriatic plaques **and** • One of the following: ○ Patient has a moderate to severe GPP flare based on **one** of the following: ▪ Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) total score ≥ 3 (moderate); **or** ▪ Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) pustulation subscore ≥ 2 (mild); **or** ▪ Erythema and pustules cover ≥ 5% of body-surface area; **or** ▪ New appearance or worsening of pustules

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Spevigo® (Spesolimab-Sbzo) (continued)	Jul. 1, 2025	the utilization of therapy (e.g., intravenous Spevigo) used for generalized pustular psoriasis (GPP) flares ○ Added list of examples of positive clinical responses to subcutaneous Spevigo therapy: preventing flares, reducing frequency of flares, prolonging time between flares, controlling signs and symptoms of GPP (e.g., pustules, erythema, pain, itching) between flares ○ Replaced references to "Stelara (ustekinumab)" with "ustekinumab" **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information	**or** ○ **All** of the following: ▪ Patient has already received one initial dose of Spevigo for a current GPP flare; **and** ▪ Documentation that the patient requires a second dose of Spevigo in order to treat persistent GPP flare symptoms including **one** of the following: – GPPPGA total score ≥ 2; **or** – GPPPGA pustulation subscore ≥ 2; **or** – Fever; **or** – Asthenia; **or** – Myalgia; **or** – Elevated C-reactive protein; **or** – Leukocytosis with peripheral blood neutrophilia [above the upper limit of normal (ULN)] **and** ▪ The second dose of Spevigo is to be administered no sooner than one week after the initial dose of Spevigo **and** • Patient is not receiving Spevigo in combination with another targeted immunomodulator [e.g., Enbrel (etanercept), Cimzia (certolizumab), Simponi (golimumab), Orencia (abatacept), adalimumab, Xeljanz (tofacitinib), Olumiant (baricitinib), Rinvoq (upadacitinib), ustekinumab, Skyrizi (risankizumab)] for treatment of the same indication; **and** • Spevigo is dosed according to U.S. Food and Drug Administration labeled dosing for GPP flares; **and** • Total dose of Spevigo does not exceed two doses per single GPP flare [**Note**: If the patient has been treated with Spevigo for a previous GPP flare, then a new (different) GPP flare may be treated with up to two doses of Spevigo]; **and** • Prescribed by a dermatologist; **and** • Authorization will be for no more than 21 days **Spevigo for subcutaneous use is proven for the treatment of generalized pustular psoriasis. Spevigo for subcutaneous use is medically necessary for the treatment of generalized pustular psoriasis when all of the following criteria are met:**

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Spevigo® (Spesolimab-Sbzo) (continued)	Jul. 1, 2025		• For **initial therapy**, **all** of the following: ○ Diagnosis of generalized pustular psoriasis (GPP) based on **both** of the following: ▪ Presence of primary, sterile, macroscopically visible pustules on erythematous base; **and** ▪ Pustulation is not restricted to the acral region or within psoriatic plaques **and** ○ **Both** of the following: ▪ Used to prevent GPP flares; **and** ▪ Patient is not currently experiencing a GPP flare **and** ○ **One** of the following: ▪ Patient has previously been treated with intravenous **Spevigo** for a GPP flare; **or** ▪ **All** of the following: – Patient has **not** previously been treated with intravenous **Spevigo** for a GPP flare; **and** – During the previous 12 months prior to initiating subcutaneous Spevigo the patient has had one or more moderate to severe GPP flares based on **one** of the following: • Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) total score ≥ 3 (moderate); **or** • Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) pustulation subscore ≥ 2 (mild); **or** • Erythema and pustules cover ≥ 5% of body-surface area; **or** • New appearance or worsening of pustules **and** – Prescriber attests that the patient has experienced flares of a severity and/or frequency such that they would clinically benefit from prophylactic therapy with subcutaneous Spevigo **and** ○ Patient is not receiving Spevigo in combination with another targeted immunomodulator [e.g., Enbrel (etanercept), Cimzia (certolizumab),

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Spevigo® (Spesolimab-Sbzo) (continued)	Jul. 1, 2025		Simponi (golimumab), Orencia (abatacept), adalimumab, Xeljanz (tofacitinib), Olumiant (baricitinib), Rinvoq (upadacitinib), ustekinumab, Skyrizi (risankizumab)] for treatment of the same indication; **and** ○ **One** of the following: ▪ Spevigo is to be administered subcutaneously as a single loading dose for a patient that is not following treatment of a GPP flare with intravenous Spevigo; **or** ▪ Prescriber attestation that the patient or caregiver is not able to be trained or is physically unable to administer maintenance subcutaneous Spevigo; prescriber must submit explanation **and** ○ Spevigo is dosed according to U.S. Food and Drug Administration labeled dosing for treatment of GPP when not experiencing a flare; **and** ○ Prescribed by a dermatologist; **and** ○ **For subcutaneous use not following treatment of GPP flare with intravenous Spevigo**: Authorization will be for one loading dose ○ **For subcutaneous use following treatment of GPP flare with intravenous Spevigo**: Initial authorization will be for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Documentation of positive clinical response to subcutaneous Spevigo therapy [e.g., preventing flares, reducing frequency of flares, prolonging time between flares, controlling signs and symptoms of GPP (e.g., pustules, erythema, pain, itching) between flares]; **and** ○ Reduction in the utilization of therapy (e.g., intravenous Spevigo) used for GPP flares; **and** ○ Patient is not receiving Spevigo in combination with another targeted immunomodulator [e.g., Enbrel (etanercept), Cimzia (certolizumab), Simponi (golimumab), Orencia (abatacept), adalimumab, Xeljanz (tofacitinib), Olumiant (baricitinib), Rinvoq (upadacitinib), ustekinumab, Skyrizi (risankizumab)] for treatment of the same indication; **and** ○ Prescriber attestation that the patient or caregiver is not able to be trained or is physically unable to administer maintenance subcutaneous Spevigo; prescriber must submit explanation; **and**

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Spevigo® (Spesolimab-Sbzo) (continued)	Jul. 1, 2025		○ Spevigo is dosed according to U.S. Food and Drug Administration labeled dosing for treatment of GPP when not experiencing a flare; **and** ○ Prescribed by a dermatologist; **and** ○ Authorization will be for no more than 12 months **Spevigo (spesolimab-sbzo) is unproven and not medically necessary for the treatment of the following conditions and situations:** • Administration of intravenous Spevigo in excess of 2 doses per single GPP flare • Atopic dermatitis • Crohn´s disease • Hidradenitis suppurativa • Palmoplantar pustulosis • Plaque psoriasis • Ulcerative colitis
Spevigo (Spesolimab-Sbzo) – IV and Subcutaneous Formulations (for UHCWest Only)	Jul. 1, 2025	**Coverage Rationale** • Revised coverage criteria: ○ Replaced criterion requiring: ▪ "Presence of primary, sterile, macroscopically visible pustules on *non-acral skin*" with "presence of primary, sterile, macroscopically visible pustules on *erythematous base*" ▪ "Pustulation is not restricted to the psoriatic plaques" with "pustulation is not restricted to the *acral region or within* psoriatic plaques" ▪ "The patient is not receiving Spevigo in combination with another targeted	**Generalized Pustular Psoriasis (GPP)** **Spevigo for intravenous use is medically necessary for the treatment of generalized pustular psoriasis flares when all of the following criteria are met:** • Diagnosis of generalized pustular psoriasis (GPP) based on **both** of the following: ○ Presence of primary, sterile, macroscopically visible pustules on erythematous base; **and** ○ Pustulation is not restricted to the acral region or within psoriatic plaques **and** • **One** of the following: ○ Patient has a moderate to severe GPP flare based on **one** of the following: ▪ Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) total score ≥ 3 (moderate); **or** ▪ Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) pustulation subscore ≥ 2 (mild); **or** ▪ Erythema and pustules cover ≥ 5% of body-surface area; **or** ▪ New appearance or worsening of pustules **or**

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Spevigo (Spesolimab-Sbzo) – IV and Subcutaneous Formulations (for UHCWest Only) (continued)	Jul. 1, 2025	immunomodulator" with "the patient is not receiving Spevigo in combination with another targeted immunomodulator *for treatment of the same indication*" ○ Added criterion for continuation of **subcutaneous** Spevigo therapy requiring reduction in the utilization of therapy (e.g., intravenous Spevigo) used for generalized pustular psoriasis (GPP) flares ○ Added list of examples of positive clinical responses to subcutaneous Spevigo therapy: preventing flares, reducing frequency of flares, prolonging time between flares, controlling signs and symptoms of GPP (e.g., pustules, erythema, pain, itching) between flares ○ Replaced references to "Stelara (ustekinumab)" with "ustekinumab" **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information	○ **All** of the following: ▪ Patient has already received one initial dose of Spevigo for a current GPP flare; **and** ▪ Documentation that the patient requires a second dose of Spevigo in order to treat persistent GPP flare symptoms including **one** of the following: – GPPPGA total score ≥ 2; **or** – GPPPGA pustulation subscore ≥ 2; **or** – Fever; **or** – Asthenia; **or** – Myalgia; **or** – Elevated C-reactive protein; **or** – Leukocytosis with peripheral blood neutrophilia [above the upper limit of normal (ULN)] **and** ▪ The second dose of Spevigo is to be administered no sooner than one week after the initial dose of Spevigo **and** • Patient is not receiving Spevigo in combination with another targeted immunomodulator [e.g., Enbrel (etanercept), Cimzia (certolizumab), Simponi (golimumab), Orencia (abatacept), adalimumab, Xeljanz (tofacitinib), Olumiant (baricitinib), Rinvoq (upadacitinib), ustekinumab, Skyrizi (risankizumab)] for treatment of the same indication; **and** • Spevigo is dosed according to U.S. Food and Drug Administration labeled dosing for GPP flares; **and** • Total dose of Spevigo does not exceed two doses per single GPP flare [**Note**: If the patient has been treated with Spevigo for a previous GPP flare, then a new (different) GPP flare may be treated with up to two doses of Spevigo.]; **and** • Prescribed by a dermatologist; **and** • Authorization will be for no more than 21 days **Spevigo for subcutaneous use is medically necessary for the treatment of generalized pustular psoriasis when all of the following criteria are met:** • For **initial therapy**, **all** of the following: ○ Diagnosis of generalized pustular psoriasis (GPP) based on **both** of

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Spevigo (Spesolimab-Sbzo) – IV and Subcutaneous Formulations (for UHCWest Only) (continued)	Jul. 1, 2025		the following: ▪ Presence of primary, sterile, macroscopically visible pustules on erythematous base; **and** ▪ Pustulation is not restricted to the acral region or within psoriatic plaques **and** ○ **Both** of the following: ▪ Used to prevent GPP flares; **and** ▪ Patient is not currently experiencing a GPP flare **and** ○ **One** of the following: ▪ Patient has previously been treated with intravenous Spevigo for a GPP flare; **or** ▪ **All** of the following: – Patient has **not** previously been treated with intravenous Spevigo for a GPP flare; **and** – During the previous 12 months prior to initiating subcutaneous Spevigo the patient has had one or more moderate to severe GPP flares based on **one** of the following: • Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) total score ≥ 3 (moderate); **or** • Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) pustulation subscore ≥ 2 (mild); **or** • Erythema and pustules cover ≥ 5% of body-surface area; **or** • New appearance or worsening of pustules **and** – Prescriber attests that the patient has experienced flares of a severity and/or frequency such that they would clinically benefit from prophylactic therapy with subcutaneous Spevigo **and** ○ Patient is not receiving Spevigo in combination with another targeted immunomodulator [e.g., Enbrel (etanercept), Cimzia (certolizumab), Simponi (golimumab), Orencia (abatacept), adalimumab, Xeljanz (tofacitinib), Olumiant (baricitinib), Rinvoq (upadacitinib),

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Spevigo (Spesolimab-Sbzo) – IV and Subcutaneous Formulations (for UHCWest Only) (continued)	Jul. 1, 2025		ustekinumab, Skyrizi (risankizumab)] for treatment of the same indication; **and** ○ Spevigo is dosed according to U.S. Food and Drug Administration labeled dosing for treatment of GPP when not experiencing a flare; **and** ○ Prescribed by a dermatologist; **and** ○ Authorization will be for no more than 12 months • For **continuation of therapy**, **all** of the following: ○ Documentation of positive clinical response to subcutaneous Spevigo therapy [e.g., preventing flares, reducing frequency of flares, prolonging time between flares, controlling signs and symptoms of GPP (e.g., pustules, erythema, pain, itching) between flares]; **and** ○ Reduction in the utilization of therapy (e.g., intravenous Spevigo) used for GPP flares; **and** ○ Patient is not receiving Spevigo in combination with another targeted immunomodulator [e.g., Enbrel (etanercept), Cimzia (certolizumab), Simponi (golimumab), Orencia (abatacept), adalimumab, Xeljanz (tofacitinib), Olumiant (baricitinib), Rinvoq (upadacitinib), ustekinumab, Skyrizi (risankizumab)] for treatment of the same indication; **and** ○ Spevigo is dosed according to U.S. Food and Drug Administration labeled dosing for treatment of GPP when not experiencing a flare; **and** ○ Prescribed by a dermatologist; **and** ○ Authorization will be for no more than 12 months **Spevigo (spesolimab-sbzo) is unproven and not medically necessary for the treatment of the following conditions and situations:** • Administration of intravenous Spevigo in excess of 2 doses per single GPP flare • Atopic dermatitis • Crohn´s disease • Hidradenitis suppurativa • Palmoplantar pustulosis • Plaque psoriasis • Ulcerative colitis

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
Testosterone Replacement or Supplementation Therapy	Jul. 1, 2025	**Coverage Rationale** • Revised list of applicable testosterone products; added Azmiro™ • Added language to indicate Azmiro™ (testosterone cypionate) has been added to the Review at Launch program and some members may not be eligible for coverage of this medication at this time; refer to the Medical Benefit Drug Policy titled *Review at Launch for New to Market Medications* for additional details • Revised coverage criteria for: ***Conditions Associated with a Deficiency or Absence of Endogenous Testosterone*** Initial Therapy ○ Added criterion requiring both of the following: ▪ Patient is currently on testosterone therapy ▪ One of the following: – Follow-up calculated free or bioavailable testosterone level, drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months for patients continuing testosterone therapy (i.e., on therapy for	**Azmiro™ (testosterone cypionate) has been added to the Review at Launch program. Some members may not be eligible for coverage of this medication at this time. Refer to the Medical Benefit Drug Policy titled Review at Launch for New to Market Medications for additional details.** This policy refers to the following testosterone products: • Testosterone cypionate (Azmiro™, Depo-Testosterone®) • Testosterone enanthate • Testosterone pellets (Testopel®) • Testosterone undecanoate (Aveed®) **Injectable testosterone and Testopel (testosterone pellets) are proven for replacement therapy in conditions associated with a deficiency or absence of endogenous testosterone, including primary hypogonadism (congenital or acquired) and hypogonadotropic hypogonadism (congenital or acquired).** **Injectable testosterone and Testopel (testosterone pellets) are medically necessary for replacement therapy in conditions associated with a deficiency or absence of endogenous testosterone, including primary hypogonadism (congenital or acquired) and hypogonadotropic hypogonadism (congenital or acquired), when the following criteria are met:** • **One** of the following: ○ Patient has history of **one** of the following: ▪ Bilateral orchiectomy; **or** ▪ Panhypopituitarism; **or** ▪ A genetic disorder known to cause hypogonadism (e.g., congenital anorchia, Klinefelter's syndrome) **or** ○ **All** of the following: ▪ **One** of the following: – Two pre-treatment early morning serum total testosterone levels less than 300 ng/dL (< 10.4 nmol/L) or less than the reference range for the lab, taken at separate times; **or** – **Both** of the following:

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Testosterone Replacement or Supplementation Therapy (continued)	Jul. 1, 2025	one year or longer), is within or below the normal male limits of the reporting lab – Follow-up calculated free or bioavailable testosterone level, drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is outside of upper male limits of normal for the reporting lab and the dose is adjusted ○ Removed notation indicating pre-treatment testosterone levels may require treatment to be temporarily held Continuation of Therapy ○ Added criterion requiring one of the following: ▪ Patient has history of one of the following: – Bilateral orchiectomy – Panhypopituitarism – A genetic disorder known to cause hypogonadism (e.g.,	• Patient has condition that may cause altered sex-hormone binding globulin (SHBG) (e.g., thyroid disorder, HIV disease, liver disorder, diabetes, obesity); **and** • **One** pre-treatment calculated free or bioavailable testosterone level less than 50 pg/mL (< 5 ng/dL or < 0.17 nmol/L) or less than the reference range for the lab **or** – **Both** of the following: • Patient is currently on testosterone therapy; **and** • **One** of the following: ○ Follow-up calculated free or bioavailable testosterone level drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year), or 12 months for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is within or below the normal male limits of the reporting lab; **or** ○ Follow-up calculated free or bioavailable testosterone level drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year), or 12 months for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is outside of upper male limits of normal for the reporting lab and the dose is adjusted **and** ▪ Patient was male at birth; **and** ▪ Diagnosis of hypogonadism **and** ○ Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** ○ Authorization will be for no more than 12 months **Injectable testosterone and Testopel (testosterone pellets) may be covered for gender-affirming hormonal therapy for transgender adults when the following criteria are met:** • **All** of the following: ○ Diagnosis of gender dysphoria, according to the current DSM (i.e.,

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Testosterone Replacement or Supplementation Therapy (continued)	Jul. 1, 2025	congenital anorchia, Klinefelter's syndrome) ▪ All of the following: – One of the following: • Two pre-treatment early morning serum total testosterone levels less than 300 ng/dL (< 10.4 nmol/L) or less than the reference range for the lab, taken at separate times • Both of the following: ○ Patient has condition that may cause altered sex-hormone binding globulin (SHBG) (e.g., thyroid disorder, HIV disease, liver disorder, diabetes, obesity) ○ One pre-treatment calculated free or bioavailable	DSM-5-TR) criteria, by a mental health professional; **and** ○ Medication is prescribed by or in consultation with an endocrinologist or a medical provider knowledgeable in transgender hormone therapy; **and** ○ Authorization will be for no more than 12 months **Compounded hormone products (e.g., pellets), including but not limited to compounded testosterone, estrogen, and progesterone pellets are not proven or medically necessary for any indication.** Compounded drugs, including compounded testosterone, estrogen, or progesterone pellets are not FDA approved.

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Testosterone Replacement or Supplementation Therapy (continued)	Jul. 1, 2025	testosterone level less than 50 pg/mL (< 5 ng/dL or < 0.17 nmol/L) or less than the reference range for the lab – Patient is currently on testosterone therapy – Patient was male at birth – Diagnosis of hypogonadism ○ Removed criterion requiring: ▪ Follow-up total serum testosterone level, drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is within or below the normal male limits of the reporting lab ▪ Follow up total serum testosterone level, drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Testosterone Replacement or Supplementation Therapy (continued)	Jul. 1, 2025	for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is outside of upper male limits of normal for the reporting lab and the dose is adjusted ▪ Patient has a condition that may cause altered sex-hormone binding globulin (SHBG) (e.g., thyroid disorder, HIV disease, liver disorder, diabetes, obesity) ***Gender-Affirming Hormonal Therapy for Transgender Adults*** ○ Removed criterion for continuation of therapy requiring one of the following: ▪ Follow-up total serum testosterone level, drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is within or below the normal male limits of the reporting lab ▪ Follow up total serum testosterone level, drawn within the past 6 months	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Testosterone Replacement or Supplementation Therapy (continued)	Jul. 1, 2025	for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is outside of upper male limits of normal for the reporting lab and the dose is adjusted ▪ Both of the following: – Patient has a condition that may cause altered sex-hormone binding globulin (SHBG) (e.g., thyroid disorder, HIV disease, liver disorder, diabetes, obesity); and – One of the following: • Follow-up calculated free or bioavailable testosterone level, drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months for	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Testosterone Replacement or Supplementation Therapy (continued)	Jul. 1, 2025	patients continuing testosterone therapy (i.e., on therapy for one year or longer), is within or below the normal male limits of the reporting lab • Follow-up calculated free or bioavailable testosterone level, drawn within the past 6 months for patients new to testosterone therapy (i.e., on therapy for less than one year) or 12 months for patients continuing testosterone therapy (i.e., on therapy for one year or longer), is outside of upper male limits of normal for the reporting lab and the dose is adjusted **Applicable Codes** • Added HCPCS code J1072	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Testosterone Replacement or Supplementation Therapy (continued)	Jul. 1, 2025	• Removed list of maximum dosage requirements and maximum allowed frequencies; refer to the Medical Benefit Drug Policy titled *Maximum Dosage and Frequency* **Supporting Information** • Updated *References* section to reflect the most current information	
Ustekinumab	Jul. 1, 2025	**Title Change** • Previously titled *Stelara® (Ustekinumab)* **Coverage Rationale** • Replaced references to "Stelara" with "ustekinumab" • Added language to indicate: ○ This policy refers to the following ustekinumab products: ▪ Otulfi® (ustekinumab-aauz) ▪ Pyzchiva® (ustekinumab-ttwe) ▪ Selarsdi™ (ustekinumab-aekn) ▪ Stelara® (ustekinumab) ▪ Steqeyma® (ustekinumab-stba) ▪ Wezlana™ (ustekinumab-auub) ▪ Yesintek™ (ustekinumab-kfce) ▪ Any FDA-approved ustekinumab biosimilar not listed here* ○ *Any U.S. Food and Drug Administration approved and	This policy refers to the following ustekinumab products for injection. Ustekinumab products for self-administered subcutaneous injection are obtained under the pharmacy benefit. • Otulfi (ustekinumab-aauz) • Pyzchiva (ustekinumab-ttwe) • Selarsdi (ustekinumab-aekn) • Stelara (ustekinumab) • Steqeyma (ustekinumab-stba) • Wezlana (ustekinumab-auub) • Yesintek (ustekinumab-kfce) • Any FDA-approved ustekinumab biosimilar product not listed here Refer to the policy for complete details.

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Ustekinumab (continued)	Jul. 1, 2025	launched ustekinumab biosimilar product not listed by name in this policy will be considered non-preferred until reviewed by UnitedHealthcare ***Preferred Product*** ○ Steqeyma (ustekinumab-stba) and Yesintek (ustekinumab-kfce) are the preferred ustekinumab products; coverage will be provided for Steqeyma and Yesintek contingent on the coverage criteria in the *Diagnosis-Specific Criteria* section [of the policy] ○ Coverage for Otulfi, Pyzchiva, Selarsdi, Stelara, Wezlana, or other non-preferred ustekinumab products will be provided contingent on the criteria in [the *Preferred Product Criteria*] section [of the policy] and the coverage criteria in the *Diagnosis-Specific Criteria* section [of the policy] ○ In order to continue coverage, members already on Otulfi, Pyzchiva, Selarsdi, Stelara, Wezlana, or other non-preferred ustekinumab products will be required to change therapy to Steqeyma or Yesintek unless they meet the criteria in [the *Preferred*	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Ustekinumab (continued)	Jul. 1, 2025	*Product Criteria*] section [of the policy] ***Preferred Product Criteria*** ○ Treatment with Otulfi, Pyzchiva, Selarsdi, Stelara, Wezlana, or other non-preferred ustekinumab biosimilar product is medically necessary for the indications specified in this policy when **both** of the following criteria are met: ▪ One of the following: – Both of the following: • Documentation of a trial of at least 14 weeks of Steqeyma or Yesintek resulting in minimal clinical response to therapy and residual disease activity • The provider attests that in their clinical opinion, the clinical response would be expected to be superior with Otulfi, Pyzchiva, Selarsdi, Stelara, Wezlana, or other ustekinumab biosimilar	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Ustekinumab (continued)	Jul. 1, 2025	products, than experienced with Steqeyma or Yesintek – Both of the following: • Documentation of intolerance, contraindication, or adverse event to Steqeyma or Yesintek • The provider attests that, in their clinical opinion, the same intolerance, contraindication, or adverse event would not be expected to occur with Otulfi, Pyzchiva, Selarsdi, Stelara, Wezlana, or other ustekinumab biosimilar products ▪ Patient has not had a loss of a favorable response after established maintenance therapy with Steqeyma, Yesintek, or other ustekinumab products ***Non-Medical Necessity Plans*** ○ Any ustekinumab product is	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Ustekinumab (continued)	Jul. 1, 2025	to be approved contingent on the coverage criteria in the *Diagnosis-Specific Criteria* section [of the policy] ***Diagnosis-Specific Criteria*** ○ "Ustekinumab" will be used to refer to all ustekinumab products • Revised coverage criteria: ○ Updated list of targeted immunomodulators the patient must not be receiving in combination with ustekinumab: ***Crohn's Disease*** ▪ Added: – Entyvio (vedolizumab) – Omvoh (mirikizumab-mrkz) – Tremfya (guselkumab) ▪ Removed: – Enbrel (etanercept) – Olumiant (baricitinib) – Orencia (abatacept) – Simponi (golimumab) – Xeljanz (tofacitinib) ***Plaque Psoriasis*** ▪ Added: – Bimzelx (bimekizumab-bkzx) – Sotyktu (deucravacitinib) ▪ Removed: – Olumiant (baricitinib) – Orencia (abatacept)	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Ustekinumab (continued)	Jul. 1, 2025	– Rinvoq (upadacitinib) – Simponi (golimumab) – Xeljanz (tofacitinib) ***Psoriatic Arthritis*** ▪ Added: – Bimzelx (bimekizumab-bkzx), Xeljanz XR ▪ Removed: – Olumiant (baricitinib) ***Ulcerative Colitis*** ▪ Added: – Entyvio (vedolizumab) – Omvoh (mirikizumab-mrkz) – Tremfya (guselkumab) – Zeposia (ozanimod) ▪ Removed: – Enbrel (etanercept) – Cimzia (certolizumab) – Olumiant (baricitinib) – Orencia (abatacept) ▪ Replaced "Xeljanz (tofacitinib)" with "Xeljanz/*Xeljanz XR* (tofacitinib)" ○ Updated list of examples of targeted immunomodulators with which the patient has been previously treated: ***Crohn's Disease*** ▪ Added: – Entyvio (vedolizumab) – Omvoh (mirikizumab-	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Ustekinumab (continued)	Jul. 1, 2025	mrkz) – Tremfya (guselkumab) ***Plaque Psoriasis*** ▪ Added: – Bimzelx (bimekizumab-bkzx) – Sotyktu (deucravacitinib) ▪ Removed: – Orencia (abatacept) ***Psoriatic Arthritis*** ▪ Added: – Bimzelx (bimekizumab-bkzx) – Xeljanz XR ▪ Removed: – Olumiant (baricitinib) ***Ulcerative Colitis*** ▪ Added: – Entyvio (vedolizumab) – Omvoh (mirikizumab-mrkz) – Skyrizi (risankizumab) – Tremfya (guselkumab) – Xeljanz XR – Zeposia (ozanimod) ○ Replaced criterion for continuation of therapy for the treatment of Crohn's disease and ulcerative colitis requiring "*Stelara* is to be *subcutaneously* administered 8 weeks after the initial	

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Ustekinumab (continued)	Jul. 1, 2025	intravenous dose" with "*ustekinumab* is to be administered 8 weeks after the initial intravenous dose" **Applicable Codes** • Added HCPCS codes C9399, J3490, J3590, Q5137, Q5138, Q9996, Q9997, Q9998, and Q9999 **Supporting Information** • Added *CMS* section • Updated *Background*, *FDA*, and *References* sections to reflect the most current information	
Viltepso® (Viltolarsen)	Jul. 1, 2025	**Coverage Rationale** • Revised coverage criteria for initial therapy; added criterion requiring **one** of the following: ○ The patient has not previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of Duchenne muscular dystrophy (DMD) ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD ▪ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional	**Viltepso (viltolarsen) may be covered for the treatment of Duchenne muscular dystrophy (DMD) in patients who meet all of the following criteria:** **Initial Therapy** • Diagnosis of Duchenne muscular dystrophy by, or in consultation with, a neurologist with expertise in the diagnosis of DMD; **and** • Submission of medical records (e.g., chart notes, laboratory values) confirming the mutation of the DMD gene is amenable to exon 53 skipping; **and** • **One** of the following: ○ Submission of medical records (e.g., chart notes, laboratory values) confirming that the patient has a 6-Minute Walk Test (6MWT) ≥ 300 meters while walking independently (e.g., without side-by-side assist, cane, walker, wheelchair, etc.) prior to beginning Viltepso therapy; **or** ○ **Both** of the following: ▪ Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** ▪ **One** of the following: – Patient has achieved a score of greater than 17 on the North

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Viltepso® (Viltolarsen) (continued)	Jul. 1, 2025	decline resulting from loss of muscle strength/motor ability (e.g., loss of a motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)]	Star Ambulatory Assessment (NSAA); **or** – Patient has achieved a time to rise (TTR) of less than 7 seconds **and** • **One** of the following: ○ Patient has not previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD; **or** ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD; **and** ○ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional decline resulting from loss of muscle strength/motor ability (e.g., loss of a motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] **and** • Viltepso is not used concomitantly with other exon skipping therapies for DMD [e.g., Amondys (casimersen), Exondys 51 (eteplirsen), Vyondys 53 (golodirsen)]; **and** • Viltepso is prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** • Initial authorization will be for no more than 12 months **Continuation Therapy** • Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** • Viltepso is not used concomitantly with other exon skipping therapies for DMD [e.g., Amondys (casimersen), Exondys 51 (eteplirsen), Vyondys 53 (golodirsen)]; **and** • Prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and**

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Viltepso® (Viltolarsen) (continued)	Jul. 1, 2025		• Reauthorization will be for no more than 12 months **Unproven** **Viltepso is unproven and not medically necessary for the treatment of other forms of muscular dystrophy (e.g., Becker muscular dystrophy).**
Vyondys 53® (Golodirsen)	Jul. 1, 2025	**Coverage Rationale** • Revised coverage criteria for initial therapy; added criterion requiring **one** of the following: ○ The patient has not previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of Duchenne muscular dystrophy (DMD) ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD ▪ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional decline resulting from loss of muscle strength/motor ability (e.g., loss of a motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene	**Vyondys 53 (golodirsen) may be covered for the treatment of Duchenne muscular dystrophy (DMD) in patients who meet all of the following criteria:** **Initial Therapy** • Diagnosis of Duchenne muscular dystrophy by, or in consultation with, a neurologist with expertise in the diagnosis of DMD; **and** • Submission of medical records (e.g., chart notes, laboratory values) confirming the mutation of the DMD gene is amenable to exon 53 skipping; **and** • **One** of the following: ○ Submission of medical records (e.g., chart notes, laboratory values) confirming that the patient has a 6-Minute Walk Test (6MWT) ≥ 300 meters while walking independently (e.g., without side-by-side assist, cane, walker, wheelchair, etc.) prior to beginning Vyondys 53 therapy; **or** ○ **Both** of the following: ▪ Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** ▪ **One** of the following: – Patient has achieved a score of greater than 17 on the North Star Ambulatory Assessment (NSAA); **or** – Patient has achieved a time to rise (TTR) of less than 7 seconds **and** • **One** of the following: ○ Patient has not previously received gene therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] for the treatment of DMD; **or** ○ **Both** of the following: ▪ Patient has previously received gene therapy [e.g., Elevidys

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
Vyondys 53® (Golodirsen) (continued)	Jul. 1, 2025	moxparvovec-rokl)]	(delandistrogene moxparvovec-rokl)] for the treatment of DMD; **and** ▪ Submission of medical records (e.g., chart notes, laboratory values) documenting a clinically meaningful functional decline resulting from loss of muscle strength/motor ability (e.g., loss of a motor milestone) since receiving gene replacement therapy [e.g., Elevidys (delandistrogene moxparvovec-rokl)] **and** • Vyondys 53 is not used concomitantly with other exon skipping therapies for DMD [e.g., Amondys (casimersen), Exondys 51 (eteplirsen), Viltepso (viltolarsen)]; **and** • Vyondys 53 is prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** • Initial authorization will be for no more than 12 months **Continuation Therapy** • Submission of medical records (e.g., chart notes) confirming that the patient is ambulatory **without** needing an assistive device (e.g., without side-by-side assist, cane, walker, wheelchair, etc.); **and** • Vyondys 53 is not used concomitantly with other exon skipping therapies for DMD [e.g., Amondys (casimersen), Exondys 51 (eteplirsen), Viltepso (viltolarsen)]; **and** • Prescribed by, or in consultation with, a neurologist with expertise in the treatment of DMD; **and** • Dosing is in accordance with the United States Food and Drug Administration approved labeling; **and** • Reauthorization will be for no more than 12 months **Unproven** **Vyondys 53 is unproven and not medically necessary for the treatment of other forms of muscular dystrophy (e.g., Becker muscular dystrophy).**

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
White Blood Cell Colony Stimulating Factors	Jul. 1, 2025	**Coverage Rationale** • Revised list of applicable short-acting filgrastim agents; added Nypozi • Added language to indicate: ○ Coverage for Nypozi will be provided contingent on the criteria in the *Preferred Product Criteria* section [of the policy] and the coverage criteria in the *Diagnosis-Specific Criteria* section [of the policy] ○ In order to continue coverage, members already on Nypozi will be required to change therapy to Zarxio or Nivestym unless they meet the criteria in the *Preferred Product Criteria* section [of the policy] ○ Treatment with Nypozi is medically necessary for the indications specified in the policy when one of the following is met: ▪ Both of the following: – History of a trial of adequate dose and duration of Zarxio or Nivestym, resulting in minimal clinical response – Physician attests that, in their clinical opinion, the clinical response would be expected to be	This policy refers to the following white blood cell colony stimulating factors (CSFs): • Long-acting pegfilgrastim agents: ○ Fulphila® (pegfilgrastim-jmdb) ○ Fylnetra® (pegfilgrastim-pbbk) ○ Neulasta® (pegfilgrastim) ○ Nyvepria™ (pegfilgrastim-apgf) ○ Udenyca® (pegfilgrastim-cbqv) ○ Stimufend® (pegfilgrastim-fpgk) ○ Ziextenzo® (pegfilgrastim-bmez) • Short-acting filgrastim agents: ○ Granix® (tbo-filgrastim) ○ Neupogen® (filgrastim) ○ Nivestym® (filgrastim-aafi) ○ Nypozi™ (filgrastim-txid) ○ Releuko® (filgrastim-ayow) ○ Zarxio® (filgrastim-sndz) • Leukine® (sargramostim) • Rolvedon™ (eflapegrastim-xnst) • Any FDA-approved white blood cell colony stimulating factor product not listed here Refer to the policy for complete details.

Revised			
Policy Title	**Effective Date**	**Summary of Changes**	**Coverage Rationale**
White Blood Cell Colony Stimulating Factors (continued)	Jul. 1, 2025	superior with Nypozi than experienced with Zarxio or Nivestym ▪ Both of the following: – History of intolerance, contraindication, or Adverse Event to Zarxio or Nivestym – Physician attests that, in their clinical opinion, the same intolerance, contraindication, or Adverse Event would not be expected to occur with Nypozi ○ Nypozi is proven and medically necessary for the following indications when the criteria listed in the policy are met: ▪ Acute myeloid leukemia (AML) induction or consolidation therapy ▪ Bone marrow/stem cell transplant ▪ Hematopoietic syndrome of acute radiation syndrome ▪ Primary prophylaxis of chemotherapy-induced Febrile Neutropenia (FN) ▪ Secondary prophylaxis of Febrile Neutropenia (FN) ▪ Severe chronic neutropenia (SCN)	

Revised

Policy Title	Effective Date	Summary of Changes	Coverage Rationale
White Blood Cell Colony Stimulating Factors (continued)	Jul. 1, 2025	▪ Treatment of Febrile Neutropenia **Supporting Information** • Updated *Background* and *FDA* sections to reflect the most current information	

Retired

Policy Title	Effective Date	Summary of Changes
Zulresso® (Brexanolone)	Jun. 1, 2025	• Retired policy; Zulresso® (brexanolone) no longer requires clinical review

General Information

The inclusion of a health service (e.g., test, drug, device, or procedure) in this bulletin indicates only that UnitedHealthcare is adopting a new policy and/or updated, revised, replaced, or retired an existing policy; it does not imply that UnitedHealthcare provides coverage for the health service. Note that most benefit plan documents exclude from benefit coverage health services identified as investigational or unproven/not medically necessary. Physicians and other health care professionals may not seek or collect payment from a member for services not covered by the applicable benefit plan unless first obtaining the member's written consent, acknowledging that the service is not covered by the benefit plan and that they will be billed directly for the service.

Note: The absence of a policy does not automatically indicate or imply coverage. As always, coverage for a health service must be determined in accordance with the member's benefit plan and any applicable federal or state regulatory requirements. Additionally, UnitedHealthcare reserves the right to review the clinical evidence supporting the safety and effectiveness of a medical technology prior to rendering a coverage determination.

UnitedHealthcare respects the expertise of the physicians, health care professionals, and their staff who participate in our network. Our goal is to support you and your patients in making the most informed decisions regarding the choice of quality and cost-effective care, and to support practice staff with a simple and predictable administrative experience. The Medical Policy Update Bulletin was developed to share important information regarding changes to our Medical Policies and Medical Benefit Drug Policies. When information in this bulletin conflicts with applicable state and/or federal law, UnitedHealthcare follows such applicable federal and/or state law.

Policy Update Classifications

New

New clinical coverage criteria have been adopted for a health service (e.g., test, drug, device, or procedure)

Updated

An existing policy has been reviewed and changes have not been made to the clinical coverage criteria; however, items such as the clinical evidence, FDA information, and/or list(s) of applicable codes may have been updated

Revised

An existing policy has been reviewed and revisions have been made to the clinical coverage criteria

Replaced

An existing policy has been replaced with a new or different policy

Retired

The health service(s) addressed in the policy are no longer being managed or are considered to be proven/medically necessary and are therefore not excluded as unproven/not medically necessary services, unless coverage guidelines or criteria are otherwise documented in another policy

The complete library of UnitedHealthcare Medical Policies and Medical Benefit Drug Policies is available at **UHCprovider.com/policies** > For Commercial Plans > Medical & Drug Policies.